

Airway Obstruction / Foreign Body / Choking

Common History

- Elderly patients with sudden coughing, choking, respiratory difficulty
- Patients with respiratory distress who are sedated from drug or alcohol use
- Patients who have had recent medical sedation
- Patients with stridor or sudden onset of wheezing
- Patients with a history of stroke, seizures, dementia, Parkinson's disease, and other neuromuscular diseases

Common Causes of Airway Obstruction in Adults

- Aspirated food -- meat, vegetables, bones
- Aspirated foreign bodies -- dental or medical appliances, teeth
- Facial or neck tumors
- Certain severe dental infections (Ludwig's angina)
- Rapidly expanding hematoma in the neck
- Laryngeal burns

EMT/AEMT

- Follow General Medical Care Guideline
- Allow patient to stay in position of comfort

If patient not moving air and conscious

- Heimlich maneuver or abdominal thrusts
 - Chest thrusts if patient is obese or pregnant
- Continue until foreign body is expelled or moved, allowing respirations

If patient not moving air and/or unconscious

- Start CPR
- Visualize oral cavity and remove foreign bodies
- BVM with OPA/NPA
- Reassess airway periodically and attempt to clear

If unable to ventilate patient

- Attempt to clear airway with chest thrusts airway clearance / suction
- Do not use Supraglottic Airway Device

Paramedic

Unconscious but able to ventilate

- Direct Laryngoscopy and foreign body removal with suctioning
- Intubate with ETT and capnography (if available)
- Do not use supraglottic airway device unless no foreign body is visualized from the mouth to the vocal cords with the direct laryngoscope

Unable to ventilate

- Direct visualization with laryngoscope
- Remove foreign body with Magill forceps
- Surgical cricothyrotomy